

**DEPARTMENT OF MEDICAL ONCOLOGY,
HEMATOLOGY AND BMT**

DISCHARGE SUMMARY

Name	Mr. SACHIDANAND SINHA	Age/Sex	78/M
IPD No	24/48121	UHID	667014
Admission Date	11/Nov/2024	Discharge Date	11/Nov/2024
Consultant	DR. PRAVEEN KUMAR BANSAL	Room	GW-MCI-1116-18
Panel	SBI General Insurance Co		

FINAL DIAGNOSIS:

- Metastatic Carcinoma Prostate (Advanced Stage, (Bladder, rectum+), Mediastinal, abdominal LN+, Bone mets, Adenocarcinoma
- Started on ADT + Enzalutamide 160 mg + Zoledronic acid 4mg w.e.f. 10/01/2024
- Received 4th dose

ON ADMISSION

Physical Examination: No cyanosis, oedema, pallor

Pulse Rate : 84 bpm, regular
BP : 100/60 mm Hg
JVP : Not raised
CVS : S1 S2 normal
Chest : B/L Clear
Abdomen : Soft, no Organomegaly
Weight : 58.6 Kg
Height : 160 cms
BSA : 1.6m²

Previous history:

A 76 years old male patient, already diagnosed case of Metastatic Ca prostate, Adenocarcinoma, Advanced stage, (Bladder, rectum +), Mediastinal, abdominal LN+, Bone mets. (Diagnosed in Metro hospital)

CECT Abdomen: suggestive of enlarged prostate with irregular outline, showing heterogenous contrast enhancement encroaching inside Urinary bladder lumen with diffuse thickening of UB wall, mild increased vascularity with extensions and multiple enlarged bilateral iliac and pre and paraaortic lymph nodes.

PSMA PET: Increased PSMA expression with prostatomegaly involving bilateral seminal vesicles and urinary bladder and rectum. Multiple mediastinal and abdominopelvic lymph nodes- metastatic (with PSMA) Multiple skeletal and marrow mets in Rt femur.

HPE: Prostate biopsy suggestive of Adenocarcinoma (Acinar type)

Upper GI Endoscopy (08/11/2023): S/o esophagitis, gastritis, RUT - negative for H pylori.

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Colonoscopy (08/11/2023): S/o Internal haemorrhoids, IC junction stricture (? Cause), scope could not be negotiated into ileum.

Terminal Ileal Biopsy (08/11/2023): Moderate Chronic Non specific Inflammation.

TRUS guided 12 core Prostate Biopsy (28/12/2024): s/o Adenocarcinoma, Acinar Type.

PSA: 57.1 (5/1/2024) Testosterone: 433 (5/1/2024)

Patient was planned for ADT+ Enzalutamide+ Inj. Zoledronic acid 4mg

Patient was admitted on 10/1/2024, in oncology day received ADT (Inj. Leuprolide 22.5mg) + Enzalutamide 160mg OD + Inj. Zoledronic acid 4mg.

PSA Total (10/04/2024) - 0.058

Leuprolide 22.5 mg + Zoledronic Acid 4 mg
Received 2nd dose on 10/04/2024

Leuprolide 22.5 mg + Zoledronic Acid 4 mg
Received 3rd dose on 17/07/2024

2D Echo (17/07/24)-No RWMA, LVEF-60%

Audiometry (17/07/24)-Right ear sloping SNHL. Left ear mild to moderate SNHL.

SISI-B/L Negative <25% retrocochlea pathology.

S. PSA (17/07/24)-0.036

2D Echo (17/07/24)-No RWMA, LVEF-60%

Audiometry (17/07/24)-Right ear sloping SNHL. Left ear mild to moderate SNHL.

SISI-B/L Negative <25% retrocochlea pathology.

2D Echo (11/11/24)-No RWMA, LVEF-55-60%

S. PSA (11/11/24)-0.014

Leuprolide 22.5 mg + Zoledronic Acid 4 mg
Received 4th dose on 11/11/2024

HOSPITAL COURSE:

Mr. SACHIDANAND SINHA, 77 yrs. old M, patient case of Metastatic Carcinoma Prostate was admitted under oncology department for 4th scheduled dose on 11/11/24.



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Patient was evaluated and relevant investigations showed:

CBC: Hb-10.6, TLC-9490, platelet count-3.18

KFT: Urea- 20, creatinine- 0.8, Na+- 126, K+-4.2, Ca-8.5

LFT: Total Bilirubin- 0.4, SGOT- 18, SGPT - 10

After stabilization patient received 4TH dose as mentioned below:

Inj. Leuprolide 22.5 mg I/M Stat

Inj. Zoledronic Acid 4mg IV in 100 ml NS over 30 minutes

20 ml NS IV Flush

Now patient is being discharged in a stable condition with follow up advice.

TREATMENT ADVISED ON DISCHARGE:

S. No	Drug Name	Dose	Frequency	Remarks	Days
1.	Tab. Enzamide	160 mg	Once a day	After Breakfast 25glt	Continue
2.	Tab Wysolone	5mg	Once daily	After food	Continue
3.	Tab. VRPZ-DSR	1 Tab	Twice a day	Before Breakfast and dinner	3 months
4.	Tab. Megha	160 mg	Once a day	After Breakfast	3 months
5.	Cap. VRX Gold	1 Cap	Twice a day	After Breakfast and dinner	3 months
6.	Picrogard Powder	2 scoops	Thrice a day	In 1 glass of milk	3 months
7.	Syp. Picmegfin	6 TSF	At Night	After Dinner	3 months
8.	Tab. Tremasine	1 Tab	Twice a day	After Breakfast & Dinner	3 months
9.	Tab. Vrzcal	1 Tab.	Once a day	After Breakfast	3 months
10.	Tab. Ecosprin	75 mg	At Night	After Dinner	Continue

Do not discontinue the above mentioned medicines unless advised by a doctor.

DIET ADVISED: Neutropenic high protein Diet.

OTHER ADVICE:

- Rest for one week.
- Continue Antihypertensive medicines as advised.

FOLLOW-UP: To review on 11/02/2025 in Medical Oncology OPD for ADT + Zoledronic acid with fresh CBC, KFT & PSA reports (09:00 AM to 02:00 PM).